

PRIORITIZATION

NCLEX STUDY GUIDE



Organized for study and printing | 12 content pages

Prioritization

1. **Maslow's Pyramid:** The most important thing is that physiological go first than psychosocial. Ex: Pain is psychosocial, no es priority except:

- Chest pain
- Lower back pain
- **Epigastric Pain**
- ★ Pancreatitis
- ★ Pre-eclampsia eclampsia
- ★ Addisonian crisis
- ★ Diabetes ketoacidosis

2. **A, B, C, D**

A = Airway

1. STRIDOR = Priority # 1

A. **Epiglottitis** (bacterial infection by haemophiles Influenza) **Signs:**

- **Drooling** (administer atropine)
- Do not introduce anything through mouth.
- **Tracheostomy at bed site**

B. **Anaphylaxis** (allergies reaction to food, medication....)

- **Hives** (ronchas) = **early sign and keyword**

C. **Face Burn** (close space)

- **Singed nasal hair or eyelashes**
- **Sooty or carbonaceous sputum**
- **Coughing Charcoal**

Note: mouth blister = herpes simple **no priority**

D. Thyroidectomy

- Decrease Ca
- Laryngeal Spasms (Tracheostomy set at bed site)
- Tetany (paresthesia, Chvostek, Trousseau)
- **Expected hoarseness during first 1 week** (ronquera) (se prioriza en todos menos aquí) only is priority if longer than 1 week or symptom worsening.
- Bleeding behind the neck and SOB

Note: increase swallowing is amygdalotomy.

E. Tracheostomy dislodged

- Ambu bag respirations the **1st 72 hours**, and activate code
- **After 72 hours** reintroduce obturator or curve tons.

Board: se le dislodgment after 4 days what to do? **Curve tons and reintroduce obturator.** If you can't do it, then activate code.

F. Dysphagia or foreign body aspiration

- Potatoes voice or muffler voice
- Child speak in word, no sentences.

G. Superior vena cava syndrome= Life threatening

- Oncologic Emergency: Mediastinal or lungs cancer
- Symptoms: Early sign.
 - Edema around the eyes.
 - Stoke's sign = Blouse collar tightness sensation
- Late sign:
 - Edema in upper extremities
 - SOB
 - Stridor

Board: Pt with lung cancer and now the wedding ring not le sirve = Superior vena cava syndrome

Board: Child with hotdog obstruction, mom constata que tiene stridor, or can't talk

H. Newborn (airway)

- Seesaw respiration
- Grunting respiration
- Nasal flatting
- Subcostal and intercostal retraction

No airway, It's Cardiovascular Problem = Circumoral or perioral cyanosis

2. Breathing**1. Respiratory Center depression**

- Opioid (Exe morphine) If 8 RR Stop.
- Magnesium Sulfate (MaSo4)
- Phenobarbital = anxiolytic and (med de election for seizures in OB and newborn)
- Lidocaine
- Benzo

2. Cervical spinal lesions

- C3-C5 = Priority C4 bc diaphragm

Note: C1- C2- C3 = black code, they are not priority (bc pt dying)

3. Muscles weakness

- A. Guillain Barre = Ascending paralysis. Principal cause: respiratory infection history: Ex: campylobacter jejune infection or influenza, or influenza vaccine, and Gastroenteritis
- Priority if respiratory compromise.

- B. Myasthenia Gravis = Priority if SOB = Proximal to distal paralysis = Ptosis

Causa = Thymoma = Tumor of the thymus and organ of the lymphatic system located in the chest behind the breastbone

- Priority for medication and the disease itself

- Neostigmine
- Physostigmine

- Give 30 Min before meals, bc el Pt se ahoga.

C. Botulism (comida enlatada) **canned food bulging or depress**

- SOB
- Ptosis
- Paralysis respiratory muscles

4. Chest Trauma

A. Pneumothorax Spontaneous

- **Skin crepitus** in chest or neck.
- **Decrease or absent unilateral Breath Sounds.**

B. Tension Pneumothorax

- **Tracheal deviation to the opposite side**

C. Open Pneumothorax

- **Sucking sound**

D. Flail Chest = more than 3 consecutive ribs broken

- **Paradoxal respiration**
- **Decrease breath sound bilateral**

F. Chest Tube Priority if:

Draining collection	Water seal control	Suction control
• More than 100 ml of drainage in a drainage collection after 24h, bc in the first 24h it is expected 500-600 ml. • Bright red drainage	• Continuous bubbling in water seal = (air leaking) = Clamp • Stop tidaling or fluctuation in a water seal chamber = lung re-expansion or kinking or obstruction of tube, or Machine malfunction = Call Dr	• intermittent vigorous blubbing

G. Mechanical Ventilation

- High pressure alarm = Patient
- Low pressure Alarm = Machine
- **Assess Pt first/Ambu bag respiration**

5. Acute pulmonary Edema

- Pink frothy sputum
- Ascending crackles more than ½ of the lung (bilateral)

6. Asthma bronchial

- Is priority when status asthmatic:
- Silence chest = Sudden relive of wheezing
- Worsening asthma condition/ SOB
- Paradoxal Pulse

7. Pulmonary Embolism

History of:

- A. DVT
- B. Long bone fracture (femur or Hip) = chest petechiae's is the keyword bc = fat embolism
- C. CVC air embolism

Signs of Pulmonary Embolism

- SOB, Tachypnea, cyanosis
- Chest pain
- Hemoptysis
- Tachycardia

8. O2 Saturation less than 95% always priority less than 90%

- Except COPD = 88-92%

3. Circulation

1. Chest Pain

- Heart burn

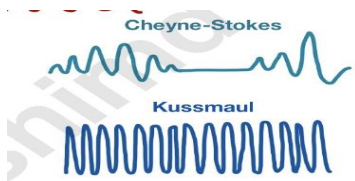
2. Epigastric Pain

- a. MI (hasta que se demuestre lo contrario)
- b. Pre-Eclampsia – Eclampsia.
- Board: Pregnant + epigastric pain = Priority bc va a cambiar de pre-eclampsia a eclampsia
- c. Addisonian crisis = increase Ca + K and decrease todo lo otro = decrease PB = hypoxemic shock = hypo + tachy

Un pt with Addison disease que ahora le duela la barriga= call Dr bc esta teniendo una Addisonian crisis.

- d. Diabetes Ketoacidosis = type 1 = type of metabolic acidosis.
- e. Fruity Odor respiration
- f. Kussmaul respiration

Note of Board: te pone dos graficas de respiraciones, kussmaul y cheyne-Stokes (conocida como la respiración de la muerte, elegir el kussmaul bc sugiere metabolic acidosis que es la periority, el Cheyne-Stoke = Black code.



3. Ventricular arrhythmias

- a. PVCs
 - 1-2 PVCs after infarction
 - 2-3 in a row

- Multifocal PVCs
- More than 6 in 1h
- b. Ventricular Tachycardia = first to do is check Pulse. **With pulse** (stable) = give medication. (Unstable) = cardioversion and **Without pulse** = defibrillation + CPR
- c. Ventricular Fibrillation = **#1 Priority** bc no Cardiac output

Note: Atrial Flutter and Atrial Fib no son priority unless you have a complication like **embolism** = slurred speech = se te fue un embolo del corazon para el brain y ahora tienes un **stroke**. Priorisas la compliacion como es SOB and Slurred speech.

- d. Torsade de Pointe = QT prolonged
- e. Potassium alterations
 - K ↑ peaked T wave. = is the most important
 - K ↓ U wave
- f. ↓ Ca & ↓ Mg Alteration
 - QT prolonged + Torsade de Pointe

g. Digoxin Toxicity = **Early Symptoms**

- Halo Vision (green or yellow)
- GI disturbance N/V/D

h. Thyroid Storm = Complication of hyperthyroidism and levothyroxine that is the hypothyroidism medication

- Fever + tachycardia

Board: Increase 1°F temperature in Hypertoroids or in a Pt with levothyroxine

Tx of Thyroid storm: main Tx = **Propranolol** to decrease hearth rate,

Propylthiouracil or **methimazole**, and Lugol Solution

4. **Shock or severe hypotension**

Symptoms of shock = **Decrease BP + Tachycardia**

- Hypovolemic
- Cardiogenic
- Septic

5. **Hemorrhage**

- Limb amputation
- Wound shoot on abdomen
- Ectopic pregnant rupture (bc sign of shock)

6. **Severe dehydration**

- Mas de 6-7 diarrheas
- Infant debe orinar 6-8 pampers per day, si orino 3 pampers = Priority bc suggest severe dehydration.

7. **Disseminate intravascular Coagulation (DIC) = Oozing of blood throught IV line**

- Ectopic pregnant
- Abrupted placenta
- Miss abortion
- Pre-eclampsia eclampsia

Unica patologia que tiene sangramiento y se trata con Heparin

8. Compartment Syndrome = related with (Cast, Snake bite or leg burn)

- Pain unreliable with medication
- Decrease or absent peripheral pulse

9. Peritonitis

- Sudden relief of abdomen pain.
- Board like abdomen pain radiated to the left shoulder

Note: los dolores que se preorizan son left shoulder, unless ectopic pregnancy on the right

10. Testicular Torsion: (less than 30 years) with painless lump in the testicle.

se prioriza la torsion testicular

- excruciating testicular pain
- Absent of cremasteric reflex (tickle on inner thigh) = Emergency

11. Tumor lysis Syndrome = Oncological Emergency

- Chemotherapy complication
- Ca↓, todo lo otro para arriba.

Tx = Plenty Fluid + Alopurinol.

Hay dos emergencias oncologicas que son: Tumor Lysis Syndrome and Superior Vena Cava Syndrome

12. Pericarditis

- Pericardial Friction Rub
- Chest Pain

Complication?

13. Cardiac tamponade

- Muffled hearth sound
- Paradoxal Pulse **same to Status Asthmatic**
- Jugular vein distention with clear lungs

Board: a Pt with pericarditis why is a priority? = Muffled heart sounds.

14. Lithium toxicity (therapeutic level = 0.5-1.5, **more than 1.5=Toxicity**)

- Nausea/Vomiting and diarrhea
- Fine hand tremors persistent more than 1 week or coarse hand tremors

Check all labs except Liver

same with aminoglycosides = Gentamicin, Amikacin, and Neomycin that respect liver too.

15. Innominate artery fistula (emergency) tracheostomy complication

- Tracheostomy pulsate in synchrony with heartbeat.

Se perforo la arteria, cause acute bleeding.

16. Maternity

- Pre-Eclampsia – Eclampsia

Pre-eclampsia = Hight BP + Proteinuria = Edema = Puffy finger

Eclampsia = Seizure

- Late deceleration = Fetal heart rate cae after peak of contraction = Falta O2 al bb=

stop oxytocin + left lateral + O2 + call Dr. **(STOP)**

- Variable deceleration = Cae a mas de 100 y cuando se mantiene por debajo de 60 por un segundo o mas y se repite varias veces es emergencia y es in signo muy grave
- Uterine rupture = Stop Contraction + hemorrhage
- Uterine inversion (mas grave) = das mal el massage
- Placenta abruption = emergency = very painful dark red bleeding

4. Neurologic D

. Complications of spinal Cord Injury

1. Autonomic Dysreflexia	2. Neurogenic Shock.
- Spinal Cord Injury above T6	(↓BP + Bradycardia)
- ↑BP + Bradycardia	

3. Cushing Triade (SATA)

- **↑ICP**
- Systolic Hypertension
- **Bradycardia**
- Widening pulse pressure or respiratory change

4. Stroke

a. Ischemic Thrombotic	b. Ischemic Embolic	c. Hemorrhagic
- History of TIA = Transient ischemic attack	- Hearth = cardiovascular problems	- Subaragnoidea hemorrhage = the worse headache of my life.
- Hypercholesterolemia	- Atrial flutter	- Cocaine abuse
	- Atrial Fib	- Severe HTN
	- Mechanical valve replacement	

5. Myxedema Coma

- Hypothyroidism = complication is Myxedema Coma. = neurologic symptoms
- Propylthiouracil or Methimazole = med of Hyperthyroidism

6. Hyperglycemic Hyperosmolar non ketonic stage

- Complication diabetes type 2. (+ 600 blood glucose)

7. Hospice or dying Pt

- **Priority is PAIN**

Emergency Department Triage = Ethical Theory – Utilitarianism = hacer mas con Menos

Primary Assessment A, B, C Assessment (indica si el paciente esta vivo o no)		Secondary Assessment	
		- Take vital sign - Pain assessment - Neurologic assessment - History - Completed Physical examination	
CODES			
RED CODE (emergency) Evaluate in less than 1 hour	YELLOW CODE (urgent) Evaluate 1-2 hours	GREEN CODE (Non-urgent) Evaluate 2-4 hours Walkie wounded & more stable patient	BLACK CODE No Priority No priority -Death or Near
1. All ABCD 2. Seizure 3. Eye chemical splash (bc deformante) 4. Wound shot 5. Pneumothorax 6. Flait chest 7. Cardiac Tamponade 8. Evaluate in less than 1h 9. Limb amputation	1. Fx open and complex 2. Disoclotion 3. Flesh wound 4. Fever 5. Surgical abdomen : appendicitis, pancreatitis. Cholecystitis. 6. Asthma without respiratory distress 7. Hypertension. 8. Renal Stone	1. Close Fx or Fx sola 2. Contusion 3. Abrasion (solo) 4. Minor laceration 5. Sprain 6. Cold symptom 7. Bruise on forehead (chichon) 8. Anxiety	1. Burn injury of 2nd- 3rd degree > 65% of the body. 2. Cushing head injury. 3. Decorticate & decerebrate posture (+ grave el 2nd) = ↑ICP (limit salvable is Cushing Triade) 4. Pupil fixed and midriática (dead brain) 5. Absent of oculophatic or doll reflex. = se murio el brain 6. RR less than 8 (if pregnant is a priority bc baby. 7. Deep abrasion + 65% (ex: Dog attack) 8. Spinal cord injury C1-C3

Preventions

Primary prevention	Secondary prevention	Tertiary Prevention
Healthy Pt Prevention of the initial occurrence of disease	Early identification of disease and treatment (Screening)	Reducing the extension and severity of an injury or illness (rehabilitation)
-Vaccination -Health Promotion -Education and teaching -Sun blocker use -Hearing protection -Body mechanic -Flex Knee -Pivot	-Mammography (every 3/y) -Colon cancer -Bread self-examination (after period 7 days) and post-menopausal (el mismo dia de cada mes) -Testicular examination -BP Screening -Blood sugar screening -Dental Check out	-Preventing Complication of the disease -Post stroke rehabilitation -OT/PT/SP -follow up of diabetes, A1C -Follow up of crhonic disease

-Folate in pregnant	-Skin evaluation (every 6 months) -All Tx (hip replacement) (remove cancer) -BMI screening -Pt diabetic with tx insulin	
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Delegation

RN	LPN	UAP
-Assessment New admitted/just receive -Diagnostic -IV medications -IV push • Adenosine • Dopamine -IV anticoagulants • Heparin -IV chemotherapy -Central venous catheter care -Legal issues • Refuse • Restrains/Assessment c/15-30min • Unstable Pt (just) just admitted/just receive • AMA • Concern -Vital sign of unstable Pt -Blood transfusion • Scheduled Pt los vital sign los hace la CNA -Feeding Pt with risk of aspiration (stroke and Parkinson) -Consultation -Teaching -Admission - First ambulation RN (bc teaching) -Discharge -Inter-consultation -Transfer -Care Plan	-Maintain and regulated IV -NG or PEG tube medication and feeding -Insertion of foley -tracheostomy care -traction (subirle el pie) -Appendectomy -Hernia repair -Wound care (stable Pt) -Colostomy care -Chronic stable Pt (Diabetes, Asthma, COPD, HTN) -Administration of Oral, IM, SQ, Topical medication -Ongoing Assessment. -Bowel sounds -Breath sounds -Neurologic Assessment -Reinforce teaching	-Apply restrain and remove -Vital sign stable pt -Post Mortem care -Stocking compression -Empty colostomy bag -ADLs activities -Feeding if not dysphagia -Recording I/O -Ambulate stable Pt -Skin barrier -Tap water enemas -CPR -Glucose test (acute check) -Stoking compression device -Transporting Urine sample or blood transfusion/ labs -Bladder scan Note: sample of urinary catheter is taken in the port

Floating Nurse

- No admission (no new admitted patient)
- No discharge
- No teaching
- Give Pt related with her/his area or stable Pt

ICU nurse to psychiatric que Pt se le da? **Board question**

- Alzheimer que diambula confundido por la sala
- Schizophrenic with auditory hallucinations
- **Depressed with severe asthma, with SOB wheezing and restlessness**
- Pt with overdose of Anti depressive tricyclics hace 48h

Emergency nurse and ICU nurse floating

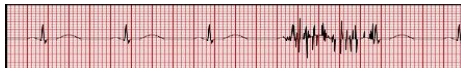
- Unstable patient

Maternity or OB RN give them:

- Wound care (bc C-section)
- Torsade de pointe (bc Mg)
- Artifacts (bc stable)
- Epidural catheter
- More stable

Board: Patient with blood transfusion and V/S schedule who take BP = CAN/ ULP Board: Maternity nurse floating in cardiology unit = give her **Torsade de pointe** patient bc = low Mg.

Note: cuando no te especifican de donde flota la Rn, usually se le da el paciente mas estable. Ex: se le da el artifact = patient moving around or device problema.

**Pediatric Nurse → Med surgical**

- Genetic diseases
- Cystic fibrosis
- Sickle cell Anemia
- Appendicitis
- Asthma
- Thrombocytopenia purpura (low platelets)

PACU nurse = 4 hours post-operative

- Hemorrhage
- Dehydration
- Electrolyte imbalance
- Malignant Hyperthermia

Outpatient nurse (trabajan en la calle)

- More stable patient

The four Phases of Emergency Management (FEMA)

MITIGATION	PREPAREDNESS	RESPONSE	RECOVERY
- Preventing future emergencies or minimizing their effect. - Include any activities that prevent an emergency. - Ex. Buying flood and fire insurance for your home. - Take place out Emergency (before and after)	- Preparing to handle an Emergency. - Rescue and Evacuation plan (Prepare plan de action). - Evacuation plan and stocking food and water. - Before an Emergency	- Responding safety to an Emergency. - Seeking shelter from tornado. - Take place during Emergency - Turning off gas and electricity.	- Action taken to return to a normal or an even safer situation. - Following Emergency
Nota: After RACE you have to EVACUATE And them PASS. one R.A.C.E you have to P.A.S.S. R = RESCUE A = ALARM C = CONFINE E = EXTINGUISH  P = PULL A = AIM S = SQUEEZE S = SWEEP R = RESCUE humans in immediate danger , should not endanger your own life A = ALARM: sound the alarm by activating a pull station alarm box and call 112 C = CONFINE the fire by closing all doors and windows E = EXTINGUISH the fire with a fire extinguisher. EVACUATE the area if the fire is too large for a fire extinguisher. ----- P = PULL the pin on the fire extinguisher A = AIM the extinguisher nozzle at the base of the fire S = SQUEEZE or press the handle S = SWEEP from side to side until the fire appears to be out Mannan THE R.A.C.E. ACRONYM AND P.A.S.S.FIRE EXTINGUISHER USE		Fire Evacuation Order: 1. Patient in intermediate danger. 2. Ambulatory patients. 3. Wheelchair isolates and cribs. 4. Bed bound patient. Note: los pacientes que no puedes mover hay que alejarlos del a ventana y taparlos con colchas	

